

NF2 Patient-Led Care — Survey & Design Rationale

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Problem

NF2 is a lifelong, multi-tumour condition. A patient's record — MRIs, audiograms, surgeries, drug trials — scatters across hospitals and systems, so assembling a coherent history and handing it to a new clinician is slow and error-prone. I am an NF2 patient designing for this gap.

Approach

Self-directed study: a bilingual patient survey (English + Chinese, distributed via Reddit / Facebook and Chinese patient communities), a literature review, and a clinical consult with a physician to pressure-test scope. Every question below was written against a specific design decision — instrument as hypothesis, not checklist.

Survey instrument — each question maps to a design decision

Q	Question theme	What it informs
1	Relationship to NF2 (patient vs caregiver)	User segmentation — identifies who actually assembles the record
2	Age range	Accessibility persona tiers (age-related vision/motor + NF2 progression)
3	Region of residence	Deployment ecosystem; need for a bilingual doctor-summary
4	Time since diagnosis	Onboarding state & default timeline density (newly-diagnosed vs veteran)
5	Sensory / physical challenges (multi)	Hard UI constraints — touch-target size, contrast, ban parallax motion
6	How history is tracked today	Validates the problem — manual/scattered records = unmet market gap
7	First reaction to a new symptom	Home-screen feature priority
8	Biggest friction explaining history to a new doctor	Summary / export layout (e.g. 60-second single-page hand-off)
9	Limitation of current community platforms	Community guardrails — filtering, anonymity, moderation
10	Pick the top 2 launch features	MVP prioritisation — lets data, not assumption, adjudicate the build
11	Optional interview sign-up	Qualitative follow-up pipeline (5–8 participants)

Scoping decisions (research judgement)

Emotion scoped out as a feature, kept as a design quality. Daily mood logging and peer support were deliberately cut from the MVP; emotional needs are met instead through tone, pacing, and visual language — and logged as a candidate research question, not a v1 feature.

Forced 2-feature ranking (Q10). Constrains respondents to surface true priority and adjudicate the contested 'research library' feature — the data decides, not the designer.

Next

Synthesise responses into personas, then prototype the core Timeline → Doctor-Summary flow in Figma.